

KERALA HEALTHCARE EXPANSION PROGRAMME

Project Nila

Strategy & Investment Plan

May 2026

PART I EXECUTIVE OVERVIEW

1. The Plan

The Program — What We Are Building

VPS Healthcare today only runs 1 unit in Kerala – the Lakeshore Hospital, Kochi and remains a sub-scale single asset in a consolidating market. Kerala's ₹35,000 Cr hospital sector is undergoing its most significant ownership transition with several large entities in a process of consolidating. The window to build a defensible Kerala network before a competitor consolidation forecloses the option set, is open today and closes within 24–36 months.

Project Nila is the response: a multi phased programme over the next 18-24 months to assemble a multi-hospital network, with equity interests in several Kerala cities including but not limited to Kochi, Thrissur, Trivandrum, Alappuzha, Malappuram, Kottayam, Kollam, Kozhikode, Palakkad and Kasaragod, and an O&M approach with several institutions where letting go of equity is difficult. The result is a ~8,300-bed Kerala-wide platform that multiplies specialist utilisation at the quaternary hub, captures the Gulf-NRI referral pipeline, and positions VPS for a credible regional-champion position.

The Investment Proposition

₹3,900 Cr Total Investment	25 Programme Hospitals	~8,300 Network Beds	₹2,090 Cr FY30 Econ. Revenue	₹495 Cr FY30 Econ. EBITDA	~20% / 2.4× Base Equity IRR / MOIC
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The base-case return of ~20% equity IRR / 2.4× MOIC over a 5-year hold is a strategic-control platform return, that meets our internal VPS IRR hurdle . Three additive upside items around VPS Lakeshore referral uplift (₹100–200 Cr estimated annual EBITDA contribution at maturity), O&M EBITDA-share above threshold, and platform multiple expansion are excluded from the base case. The cost of inaction is material: for example, if Aster or KKR acquires institutions within the existing Lakeshore entity such as Sunrise, Renai or Mother, the annual EBITDA at risk to VPS Lakeshore from lost referral volume is estimated at ₹30–60 Cr.

Structure of the Program

The program uses two entry playbooks. Playbook A (Strategic-Control): VPS acquires majority equity or wins an NCLT going-concern resolution and consolidates the hospital's economics. Playbook B (O&M): VPS operates a partner trust or mission hospital under a 15–25 year concession for a management fee, without owning equity or real-estate.

Playbook	Entry Mechanics	Capital	Value Capture	Best For
A — Strategic-Control (Equity / NCLT / JV)	Acquire 60–100% stake or NCLT going-concern resolution. Full operating control. Consolidate economics.	₹140–375 Cr per asset	Operating profit + full equity upside.	Distressed assets (SUT Royal), succession-driven sales (ex : Mother, Renai, Sunrise, Indo-American)

Playbook	Entry Mechanics	Capital	Value Capture	Best For
B — O&M Model	15–25 year concession. VPS deploys management team + modernisation capex. No equity transfer.	₹50–200 Cr capex	5–8% of gross hospital revenue as management fee + EBITDA-share above threshold.	Mission hospitals and institutions that will not sell equity

Beds & Capital by Playbook

The 25-hospital priority programme splits across the two playbooks as set out below. Playbook A capital is acquisition enterprise value or JV capital — VPS takes equity or operating control. Playbook B capital is modernisation / turnaround capex only: there is no acquisition cost, since the partner retains land, building and licence, and VPS earns a 6.5% management fee on gross revenue. All entry-stage figures are indicative and subject to due diligence.

Playbook	Hospitals	Beds	Priced Capital	What the capital represents
A — Strategic-Control (Equity / NCLT / JV)	17	~4,900	~₹2,300 Cr	5 owned ₹1,240 Cr + Pushpagiri/Sreevalsam JVs ₹200 Cr + 6 acquisitions ₹860 Cr priced at ~7× EBITDA (Daya, Ananthapuri, KM Cherian, Starcare, Mother Care, Meditrina).
B — O&M concession	8	~3,360	~₹520 Cr	Modernisation / turnaround capex only — Lourdes, Caritas, Bishop Benziger, Aswani, SR Medical, Puthiyedom, Travancore + Aventus (turnaround O&M). No acquisition cost; 6.5% management fee.
Priority total (25 hospitals)	25	~8,300	~₹2,820 Cr	≈ 72% of the ₹3,900 Cr envelope; the balance funds integration and growth capex (incl. Kakkanad greenfield), working capital and contingency.

Programme envelope ₹3,900 Cr = ₹2,150 Cr equity (55%) + ₹1,750 Cr debt (45%).

60-Day Action Track

Priority	Track	Action	Timing
Week 1	Appoint Legal DD Counsel	Mandate a Tier-1 law firm (AZB, Cyril Amarchand, or Trilegal) to lead legal due diligence across target shortlist: title searches, NCLT/CIRP status checks, promoter litigation screening, and Sec 29A self-check across VPS group entities.	Day 7
Week 1	Appoint Financial DD Advisor	Engage a Big-4 firm (PWC, or EY) to conduct quality-of-earnings, working-capital normalisation, and debt-mapping on the top-5 priority targets. Scope to include EBITDA adjustability and capex liability assessment.	Day 7
Week 2	Decide on a Lead Investment Banker	Issue a formal IB mandate to a healthcare-specialist M&A advisor (Kotak, 7i, JM Financial) to run structured deal processes, manage seller outreach, and provide valuation benchmarking across all shortlisted targets.	Day 10
Week 2	Start preparing an Operational DD Plan	Form an internal team to assess clinical governance, staffing structures, bed-utilisation patterns, and NABH/JCI compliance gaps across concession targets.	Day 14

Priority	Track	Action	Timing
Week 2	DD Data Room & Information Protocol	Define information-request lists (IRLs) per target archetype (acquisition vs O&M). Agree NDA templates with advisors; initiate preliminary document collection from willing targets.	Day 14
Week 3	Regulatory & Tax Structuring Advisor	Mandate a tax advisory firm to review optimal acquisition structures (direct purchase, slump sale, SPV, or O&M concession) for each archetype, including stamp duty, GST, IT, and FCRA implications for mission-hospital channel.	Day 21
Month 2	Debt Financing Advisor / Arranger	Appoint a debt capital markets advisor or domestic bank consortium lead (SBI Capital, ICICI Securities, or Axis Capital) to structure the ₹1,750 Cr debt envelope — term sheet parameters, security package, and covenant framework.	Day 30
Day 60	DD Progress Review & Advisor Alignment	Convene a cross-advisor alignment session: Legal, Financial, Operational, and Tax advisors present preliminary DD findings. Rank targets by risk-adjusted attractiveness. Agree priority sequence for binding offers and NBO submissions.	Day 60

PART II INVESTMENT CASE

2. Investment Thesis

Strategic Operator · Management Partner · Value Creation at Scale

Kerala's ₹35,000 Cr hospital market is consolidating faster than any single operator is positioned to capture. The programme acquires majority equity in willing sellers and NCLT assets (Playbook A) and manages mission hospitals under O&M concession (Playbook B), building a defensible 25-hospital Kerala-wide platform. Base-case: 25 active hospitals · ~₹2,090 Cr economic revenue at ~23.7% EBITDA by Year 5 · ~20% equity IRR / ~2.4× MOIC.

The Opportunity — Why Now

Succession-Stress Wave

Founders of Kerala's 1970s–90s hospital generation are ageing out. At least six priority targets are in active second-generation transition — Mother Hospital (6 children on board), Sunrise (Hafeez family, daughters already directors), Indo-American (founder ~94, Buffalo NY), Cosmopolitan (8-director board, no single controller).

NCLT / CIRP Distressed Pipeline

Several entities are in a distressed state. SUT Royal Hospital (TIHSL, Kochulloor) is in liquidation under a going-concern protocol — the cheapest plausible Trivandrum tertiary entry at an estimated EV of ₹100–180 Cr. PVS

Memorial (Kaloor, Kochi) is in CIRP with a Supreme Court challenge pending; bilateral track possible if SC upholds the resolution plan. Distressed entry delivers capacity at 40–60% discount to replacement cost.

Mission-Hospital Capital Crunch

Catholic and trust hospitals — Lourdes (600 beds, NABH-accredited), Caritas (655 beds, combined ~800 post-Matha merger), Bishop Benziger (600 beds, Kollam) — face post-COVID balance-sheet stress and modernisation capex they cannot self-fund. Their boards seek a brand-neutral operational partner with capital, Gulf credentials, and a willingness to preserve community and religious identity.

Competitive Window Dynamics

Actor	Move	Kerala Footprint
Aster DM + QCIL	India's 3rd-largest chain. They are in Kochi, Thrissur, North Kerala premium tier.	8 hospitals in Kerala (post-QCIL merger): Aster Medcity Kochi (900 beds, expanding to 962 by FY27), MIMS Kozhikode, MIMS Kottakkal, MIMS Calicut, Aster Thrissur, Wayanad Medical College, Aster MIMS Kasaragod (264 beds, opened Oct 2025), plus Aster Capital Trivandrum (454 beds, greenfield). ₹850 Cr committed to Kerala over FY26–28.
KKR / Baby Memorial	Locked North Kerala hub-and-spoke. Baby Memorial + BMH network across Kozhikode–Kannur–Kasaragod corridor.	~1,000 beds across Kozhikode (BMH flagship, 600 beds) and Kannur. Added Meitra Hospital Kozhikode (230 beds, acquired Sept 2024), locking the Kozhikode–Kannur–Kasaragod corridor. KKR total commitment to BMH now >₹5,100 Cr; targeting 3,000 beds across network. Active acquirer in the South/Central Kerala market.

Entry Strategy — Two Playbooks

	Playbook A — Strategic-Control Model	Playbook B — O&M Model
Entry	Majority equity / NCLT control. 60–100% stake or going-concern resolution.	O&M concession. 15–25 year term; no equity/title transfer.
Capital per asset	₹140–375 Cr (acquisition EV + integration capex)	₹50–200 Cr modernisation capex secured against concession term
Revenue model	Consolidated hospital P&L at VPS stake%	Management fee: 6.5% of gross hospital revenue + EBITDA-share above threshold
Exit treatment	Consolidate; exit at hospital multiple (14× base case)	Exit as services annuity (8× base case). NOT at hospital multiple.
Best for	NCLT assets, promoter exits, family succession sales	Mission hospitals (religious trusts, archdioceses) that will not sell equity

Value-Creation Engine — Six Levers

Lever	Mechanism	Typical Uplift	Timeline
Case-mix upgrade	Shift to specialist/super-specialist procedures via VPS Lakeshore tumour boards, cath labs,	+5–8 % EBITDA	12–18 mo

Lever	Mechanism	Typical Uplift	Timeline
	transplant protocols. Route complex cases to hub.		
ARPP management	Standardise package pricing to chain norms; eliminate under-billing; improve insurer/corporate mix.	+15–25% ARPP	3–6 mo
Utilisation uplift	Hub-spoke referral loop; specialist rotations from VPS Lakeshore; IPA/TPA empanelment.	+10–20% occupancy	6–18 mo
Procurement	Group purchasing: pharma, consumables, equipment across all Owned hospitals via VPS Lakeshore vendor framework.	+2–3 % EBITDA	1–3 mo
Payer mix	MEDISEP, CGHS, PSU, corporate TPA empanelment; reduce cash-pay dependence.	+3–5 %EBITDA	6–12 mo
Fixed-cost leverage	Revenue growth on existing infrastructure — shared Finance, HR, IT, biomedical engineering at VPS Lakeshore.	+3–6 % EBITDA	18–36 mo

Exit Pathways

Route	Description	Indicative Timeline	Value Driver
Strategic sale	Sale to a pan-India chain (Manipal, Narayana, Fortis) seeking Kerala entry via an assembled platform rather than deal-by-deal.	FY31–FY32	Platform premium: 9-asset credentialed network exits at 16–20× vs 12–14× for a single asset.
Secondary Financial Sale	Sale to a financial investor seeking a healthcare platform with visible referral corridors and mission-hospital concession annuities.	FY31–FY32	IRR-driven; platform scale and fee-stream predictability are the valuation anchors.
IPO / Burjeel consolidation	Roll into Burjeel Holdings listed vehicle (ADX) as the India asset. Alternatively, list as VPS India entity.	FY32+	Listed multiple (Aster/KIMS at 18–25× LTM EBITDA) — significant uplift vs base-case 14×.

3. Investment Memorandum

₹3,900 Cr Envelope	25 Hospitals (base)	~8,300 Network Beds	₹2,090 Cr FY30 Econ. Revenue	₹495 Cr (23.7%) FY30 Econ. EBITDA	~20% / ~2.4x Base IRR / MOIC
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The immediate next step before any capital commitment is to mandate advisors and commence due diligence. The Board is asked to approve the initiation of DD on 3-5 anchor assets: execution of NDAs, appointment of legal and financial advisors, and commencement of full diligence. No capital is deployed at this stage. Each anchor deal will return to the Board individually for approval before any LOI or binding commitment is signed.

The broader program envelope will be considered only after diligence on the anchor assets is complete and a parent funding bridge is confirmed. No delegated authority to execute transactions is sought.

The Case

Owned + NCLT entry (Renai, SUT Royal, Sunrise, Mother, Indo-American) add consolidated tertiary capacity in Kochi, Trivandrum, Thrissur and a neuro-oncology node. At VPS's economic stake, these contribute directly to network EBITDA and exit enterprise value.

Mission-hospital O&M (Lourdes, Caritas, Bishop Benziger) is structurally available to VPS as a brand-neutral, non-buyout operator. This is a fee-income business.

The O&M Plan

VPS assumes full operational authority over a partner mission hospital for a 15–25 year term, **without acquiring equity, ownership or real-estate title**. The owner (archdiocese, diocese or trust) retains 100% ownership and a Board seat. VPS deploys its management team and commits ₹50–200 Cr of modernisation capex secured against the concession term. VPS earns a management fee of **6.5% of gross hospital revenue** (midpoint of 5–8% range) plus a performance-linked EBITDA-share above a threshold.

PART III TARGET PORTFOLIO

4. Target Universe

4A. Tier-1 Priority Targets

#	Hospital	Location	Beds	Entity	Deal Structure	EV / Capex	VPS Stake	Entry Rev (₹Cr)	Entry EBITDA	Phase	Playbook
1	Renai Medicity	Kaloor, Kochi, Ernakulam	335	Partnership firm (Polakulath Group — 5 siblings)	Equity 65%	₹300 Cr	65%	₹160 Cr	8%	FY26	A
2	Sunrise Hospital	Kakkanad, Kochi, Ernakulam	200	Pvt Ltd — U85110KL2002PTC01 5726	Equity 70% (founder rollover)	₹240 Cr	70%	₹120 Cr	16%	FY27	A
3	Lourdes Hospital	Pachalam, Kochi, Ernakulam	600	Religious trust (Archdiocese of Verapoly)	O&M concession (25-yr)	₹175 Cr capex	n/a	₹230 Cr	15%	FY26	B
4	Caritas Hospital	Thellakom, Kottayam	655	Public Charitable Trust (Knanaya Catholic Archdiocese)	O&M / managed services (25-yr)	₹120 Cr capex	n/a	₹180 Cr	17%	FY27	B
5	Pushpagiri Medical College	Tiruvalla, Pathanamthitta	678	Registered Charitable Society (Syo-Malankara Archdiocese)	JV 51:49 (academic)	₹200 Cr JV capex	51%	₹210 Cr	20%	FY28	A
6	Indo-American Hospital (Brain & Spine)	Vaikom border, Alappuzha	205	Bahuleyan Charitable Foundation (Dr K. Kumar Bahuleyan)	Equity / scheme of amalgamation (85%)	₹185 Cr	85%	₹90 Cr	18%	FY27	A
7	Bishop Benziger Hospital	Kollam	600	Charitable Society (Latin Catholic Diocese of Quilon)	O&M concession (20-yr)	₹175 Cr capex	n/a	₹90 Cr	13%	FY27	B
8	SUT Royal Hospital (TIHSL)	Kochulloor, Trivandrum	400	TIHSL — under NCLT liquidation (U85110KL2008PLC02 1910)	NCLT going-concern (liquidation)	₹140 Cr	100%	₹97 Cr	4%	FY26	A
9	Mother Hospital	Pullazhi, Thrissur	300	Mother Hospital Pvt Ltd (U85110KL1988PTC00 5265)	Equity 80–100%	₹375 Cr	90%	₹150 Cr	18%	FY27	A

#	Hospital	Location	Beds	Entity	Deal Structure	EV / Capex	VPS Stake	Entry Rev (₹Cr)	Entry EBITDA	Phase	Playbook
10	Orchid Medicity	Malappuram	250	Pvt Ltd (to be verified)	Equity — strategic-control	TBD (DD)	Majority	TBD	est.	FY27	A
11	Sreevalsam Inst. of Medical Sciences	Edappal, Malappuram	350	Society / Pvt	Equity / academic JV	TBD (DD)	Majority	TBD	est.	FY28	A
12	CM Hospital	Kasaragod	120	Family (verify)	Equity — strategic-control	TBD (DD)	Majority	TBD	est.	FY28	A
13	MVJM Hospital	Perumbavoor, Ernakulam	100	Family (verify)	Equity — strategic-control	TBD (DD)	Majority	TBD	est.	FY27	A
14	Joys Hospital (W&C)	Vyttila, Kochi, Ernakulam	60	Family (verify)	Equity — bolt-on	TBD (DD)	Majority	TBD	est.	FY27	A
15	Aswani Hospital	Thrissur	300	Pvt (verify)	O&M concession	capex TBD	n/a	TBD	est.	FY27	B
16	SR Medical College	Varkala, Trivandrum	300	Society — med college	O&M (30-yr)	capex TBD	n/a	TBD	est.	FY28	B
17	Puthiyedom Hospital	Pala, Kottayam	75	Family (verify)	O&M concession	capex TBD	n/a	TBD	est.	FY28	B
18	Travancore Medical College	Mylapore, Kollam	650	Society — med college	O&M / academic JV	capex TBD	n/a	TBD	est.	FY28	B
19	Daya General Hospital	Thrissur	500	Daya General Hospital Ltd (public ltd)	Equity 75% (~7× EBITDA)	₹210 Cr	75%	₹187 Cr	16%	FY26	A
20	Ananthapuri Hospitals	Thiruvananthapuram	400	Ananthapuri Hospitals Pvt Ltd	Equity 75%	₹165 Cr	75%	₹175 Cr	13%	FY27	A
21	Dr. KM Cherian IMS	Chengannur, Alappuzha	300	Dr. KM Cherian IMS Pvt Ltd	Equity 75%	₹200 Cr	75%	₹168 Cr	17%	FY27	A
22	Starcare Hospital	Kozhikode	290	Starcare Hospital Kozhikode Pvt Ltd	Equity 75%	₹95 Cr	75%	₹106 Cr	13%	FY28	A
23	Mother Care & Health Centre	Kozhikode	250	Mother Care & Health Centre Pvt Ltd	Equity 75% (W&C)	₹50 Cr	75%	₹63 Cr	11%	FY28	A

#	Hospital	Location	Beds	Entity	Deal Structure	EV / Capex	VPS Stake	Entry Rev (₹Cr)	Entry EBITDA	Phase	Playbook
24	Meditrina Hospitals	Kollam	175	Meditrina Hospitals Pvt Ltd	Equity 75%	₹140 Cr	75%	₹181 Cr	11%	FY27	A
25	Avitis Super Specialty	Nemmara, Palakkad	180	Avitis Super Specialty Hospitals Pvt Ltd	O&M / turnaround concession	₹50 Cr capex	n/a	₹69 Cr	(4)%	FY28	B

4B. Per-Target DD Notes & Key Flags

The following notes help support future critical diligence flags that must be resolved before deal execution.

1. Renai Medicity — Kaloore, Kochi

- **Entity structure:** Partnership firm (Polakulath Narayanan Renai Medicity) — not a Pvt Ltd. Deal requires inter-sibling sign-offs from all 5 partners (P.N. Krishnadas leads, plus 4 siblings).
- **Financial opacity:** CARE rating downgraded BB+ (2020) → B (2023); issuer non-cooperating since 2022. Only FY17 public financials available (₹93 Cr revenue, ₹10 Cr PAT). Estimated current revenue ₹160–220 Cr.
- **Distress signal:** 335 operational beds vs 500 capacity. Group diversified across hotels, plantations, fisheries — possible cross-subsidy issues.
- **DD mandate:** Insist on FY23–25 audited accounts as condition of any indicative offer. Banker-channelled approach since CARE distress is the natural trigger.

2. Sunrise Hospital — Kakkanad, Kochi

- **Multi-unit network:** Sunrise operates in Kakkanad + 4–5 additional units (Erattupetta, Changaramkulam-Thrissur, Kanhangad, Kulanada-Pathanamthitta, possible PVS Hospital Kozhikode affiliation). Based on whether we want to acquire Kakkanad only or the full chain, the EV range shifts from ₹240 Cr (single unit) to ₹450–600 Cr (full network).
- **Founder-doctor dependency:** Dr P.R. Hafeez Rahman is the brand. Daughters (Taniya, Sonia) are already on the board, with some level of a succession path visible.
- **Specialties:** Strong W&C / IVF franchise. ART Act licence portability on change of control must be confirmed before signing.
- **Strategic fit:** IT-corridor anchor; complements VPS Lakeshore without cannibalising. Strong NRI / Gulf-returnee catchment.

3. Lourdes Hospital — Pachalam, Kochi

- **Trust structure:** Under Archdiocese of Verapoly (Latin Catholic — Roman Rite). Here only an O&M/concession route is viable.
- **Scale:** 500 IPD/day + 1,700 OPD/day footprint. NABH-accredited (first mission hospital in Kerala). DNB postgraduate program in 14 specialties.
- **Concession enforceability:** Legal opinion required on whether Archbishop of Verapoly can bind successors for 25 years and whether a charge can be secured against the concession term.
- **Critical path:** Based on success, this can be the proof-of-concept for the entire mission-hospital O&M playbook.

4. Caritas Hospital — Thellakom, Kottayam

- Matha Hospital merged into Caritas as 'Caritas Matha Hospital'. Combined capacity ~800 beds. Route all Matha engagement via Caritas Trust.
- **Governance:** Patron: H.E. Mar Mathew Moolakkatt (Archbishop). Chairman: Bishop Mar Joseph Pandarasseril. Here, both must approve any concession agreement.

- **Denomination:** Knanaya Catholic Archeparchy of Kottayam — small, cohesive, strong US/Gulf NRI diaspora. Different from Syro-Malabar (Lisie/Pushpagiri) and Latin Rite (Lourdes/Benziger).
- **Trust operations:** Also runs Caritas College of Nursing, Pharmacy, IMER. Affordable-care commitment must be written into concession.

5. Pushpagiri Medical College — Tiruvalla

- **JV complexity:** Medical college requires NMC/KUHS regulatory compliance. Society governance via Syro-Malankara Catholic Archdiocese — Patron Archbishop Thomas Mar Koorilos must approve.
- **Distinguish:** Pushpagiri Medical College ≠ Pushpagiri Heart Institute ≠ Cyril Baselios Catholicos Hospital — same trust, different institutions. JV is with the Medical College entity only.
- **Academic anchor:** 100 MBBS seats/year; KUHS-affiliated; NMC-recognised. NMC seat-enhancement application could add substantial non-clinical revenue.
- **Deadlock risk:** 51:49 structure with a charitable society requires unanimous consent on key capital decisions. Robust deadlock resolution (casting vote / put-call) essential in SHA.

6. Indo-American Hospital — Vaikom border, Alappuzha

- **Structure:** Bahuleyan Charitable Foundation (BCF) — not-for-profit. Simple equity acquisition not possible. Requires scheme of amalgamation with a VPS-controlled trust or trust-deed amendment.
- **Clinical asset:** 205 beds; neurosurgery/neurology specialty depth. Perfect bolt-on for Lakeshore tumour board. Day-90 unified oncology pathway viable if clinical talent retained.
- **Location** - Vaikom border, Alappuzha district

7. Bishop Benziger Hospital — Kollam

- **Denomination :** Latin Catholic Diocese of Quilon. Vatican-aligned Catholic governance; Bishop's authority over diocesan institutions is absolute.
- **Beds:** 600 beds. Capex budgeted to ₹150–200 Cr.
- **Approach:** Via Bishop Paul Antony Mullassery, Diocese of Quilon. Engage via CBCK (Catholic Bishops' Conference of Kerala).
- **Posture:** Charity-oriented where VPS can frame concession around community mission and not commercial return.

8. SUT Royal Hospital (TIHSL) — Kochulloor, Trivandrum

- **Entity :** Trivandrum International Health Services Limited (TIHSL), Kochulloor..
- **NCLT timeline:** CIRP admitted Feb 2020 → Resolution Plan rejected (failed 66% CoC threshold) → Liquidation ordered Jun 2022 → NCLAT appeal dismissed Oct 2022 → Latest IA dismissed May 2025. Liquidator: Raju Palanikkunathil Kesavan.
- **Key risks:** Sec 29A self-check required across VPS group. Competing UAE bidder risk possible(Aster QCIL or BR Life). 5-year consultant attrition during proceedings to diligence. EV ₹100–180 Cr.
- **Opportunity:** Clean-slate NCLT entry targets 24% EBITDA margin by Year 3 from a 4% entry base — highest improvement case in the portfolio.

9. Mother Hospital — Pullazhi, Thrissur

- **Governance:** Mother Hospital Pvt Ltd (U85110KL1988PTC005265). 6 directors — Dr M. Ali family, second generation already in management (Hassan Ali, Shabna Ali, Shermin Ali, Akbar Ali, Ayshabeevi).
- **Financial flag:** As per the MCA records, the paid-up capital only ₹1.34 Cr which is probably incorrect. True operating-asset value will substantially higher.
- **Compliance:** FY25 balance sheet filed. 199 employees, 10-acre campus on Thrissur-Kanjany-Vatanappally highway. EV ₹300–450 Cr.
- **Approach:** Bilateral via founder generation (Dr M. Ali / Dr Aysha Ali) with succession-monetisation framing. Co-brand 'VPS Lakeshore Mother Thrissur' for 24 months post-close to preserve clinical brand.

10. Orchid Medicity — Malappuram

- Playbook A — Equity acquisition (strategic-control). Anchor for the Malappuram corridor; operating since 1990 with 250 beds and deep community trust.
- Structure: Pvt Ltd — verify exact shareholding and family ownership split before approach. First-mover urgency is high; no other network hospital in the district.
- DD mandate: Confirm MCA filings, FY23–25 audited P&L, bed utilisation, and CIN. Assess clinical quality gaps vs VPS protocols for Day-1 integration plan.
- Approach: Direct bilateral with promoter family. Position as district anchor for VPS Kerala network with co-branding for 24 months post-close.

11. Sreevalsam Institute of Medical Sciences — Edappal, Malappuram

- Playbook A — Equity / academic JV (51:49). Planned 600-bed medical college campus with teaching infrastructure already in place; adds Malappuram scale and a medical-education income stream.
- Regulatory: NMC/KUHS-regulated — SHA must include deadlock-protection and academic governance provisions. Verify NMC recognition status and affiliation compliance.
- DD mandate: Confirm society registration, NMC Letter of Permission, student intake quota, FY23–25 revenue. Assess hospital-to-college revenue split and liabilities.
- Approach: Via academic intermediary or direct to society trustees. Position as governance upgrade and capital injection to complete expansion to full 600-bed campus.

12. CM Hospital — Kasaragod

- Playbook A — Equity acquisition (strategic-control). Closes the northern geographic gap with ~120 beds and cross-border catchment extending into Karnataka (Mangaluru corridor).
- Rationale: Only comprehensive multispecialty in the district. Cath lab and stroke rehab assets align with VPS clinical protocols. Aster MIMS Kasaragod (264 beds, Oct 2025) is now the primary competitor.
- DD mandate: Confirm family ownership structure, MCA filings, FY23–25 financials, NABH/JCI status, and bed utilisation. Assess Aster competition impact on pricing.
- Approach: Bilateral with founder family. Succession-monetisation framing; Aster's entry makes timing more urgent for promoter.

13. MVJM Hospital — Perumbavoor, Ernakulam

- Playbook A — Equity acquisition (strategic-control). ~100-bed facility on the industrial belt midpoint between Kochi and Kottayam; strong emergency and general medicine base.

- Competitive urgency: KKR-backed BMH is actively evaluating the same corridor — early execution secures an Ernakulam east presence ahead of the competition.
- DD mandate: Confirm ownership structure, MCA/RoC filings, FY23–25 P&L, bed utilisation by department, and land title. Assess operational gaps for Day-1 VPS standards.
- Approach: Founder-family bilateral with quick-close framing given BMH competitive pressure.

14. Joys Hospital (Women & Children) — Vyttila, Kochi

- Playbook A — Equity bolt-on (~60 beds, women and children). Vyttila bypass location creates a managed referral corridor for high-risk maternity and NICU overflow directly into VPS Lakeshore.
- Strategic fit: Deepens Kochi city market density and adds a dedicated W&C node that Lakeshore cannot absorb at scale. KKR/BMH Meitra acquisition makes Ernakulam consolidation more competitive.
- DD mandate: Confirm promoter family ownership, MCA filings, FY23–25 financials, NABH compliance, and specialist roster. Assess NICU capacity and obstetric case-mix.
- Approach: Direct family approach; position as VPS network affiliate with Lakeshore as the tertiary backstop for complex cases.

15. Aswani Hospital — Thrissur

- Playbook B — O&M concession. 300-bed, well-established institution since 1987 with 80 doctors on staff. Deploys VPS clinical governance, HIS, and brand at zero capital outlay.
- Strategic fit: Thickens the Thrissur cluster alongside Mother Hospital (target 9); already an inbound referral partner. Management fee and revenue-share contribution from first operating quarter.
- DD mandate: Confirm ownership structure, existing management contracts, FY23–25 P&L, accreditation status, and labour obligations. Draft O&M term sheet with 5-year initial term.
- Approach: Position as a clinical upgrade and brand elevation for the institution with no ownership dilution for promoters.

16. SR Medical College — Varkala, Thiruvananthapuram

- Playbook B — O&M concession (30-year). 300-bed teaching hospital adds academic credentials to the VPS network and establishes a Thiruvananthapuram district presence.
- Strategic fit: Academic node complementing the SUT Royal Trivandrum entry. Medical college affiliation creates a direct training pipeline into VPS group hospitals.
- Regulatory: Society-owned medical college — O&M agreement must address NMC obligations, faculty employment continuity, and student welfare provisions.
- DD mandate: Verify NMC/KUHS recognition, society registration, FY23–25 financials, existing management structure, and land encumbrances.

17. Puthiyedom Hospital — Pala, Kottayam

- Playbook B — O&M concession. ~75-bed community hospital in the Kottayam rural belt — natural spoke complementing Caritas (target 4) and Pushpagiri (target 5) in central Kerala.
- Strategic fit: O&M structure preserves existing family ownership while raising clinical and administrative standards to VPS protocols. Extends Kottayam district reach into underserved rural catchment.

- DD mandate: Confirm family ownership, FY23–25 P&L, accreditation, specialist roster, and existing debt. Draft 10-year O&M term sheet with management fee and capex obligations.
- Approach: Bilateral with family owners; position as governance upgrade without ownership change.

18. Travancore Medical College — Kollam

- Playbook B — O&M / academic JV. ~650-bed medical college hospital in Kollam; large academic backstop and geographic complement to the Bishop Benziger concession (target 7) in the same district.
- Strategic fit: Adds significant bed scale and academic affiliation in Kollam-Thiruvananthapuram corridor. Society ownership means no equity outlay — O&M or revenue-share structure only.
- Regulatory: Society-owned — O&M agreement must cover NMC faculty obligations, student welfare, and existing affiliation terms with Kerala University of Health Sciences.
- DD mandate: Verify NMC recognition, society governance, FY23–25 financials, existing liabilities, and overlap with Bishop Benziger clinical services.

4C. Tier-2 Targets

#	Hospital	Location	Beds	Entity	Deal Structure	EV / Capex	VPS Stake	Entry Rev (₹Cr)	Entry EBITDA	Phase	Playbook
1	CM Hospital	Kasaragod	~120	Family (verify)	Equity — strategic-control	TBD (DD)	Majority	TBD	est.	FY28	A
2	Sreevalsam Inst. of Medical Sciences	Edappal, Malappuram	~350	Society / Pvt (verify)	Equity / academic JV	TBD (DD)	Majority	TBD	est.	FY28	A
3	Joys Hospital (W&C)	Vyttila, Kochi, Ernakulam	~60	Family (verify)	Equity — bolt-on	TBD (DD)	Majority	TBD	est.	FY27	A
4	Orchid Medicity	Malappuram	250	Pvt Ltd (verify)	Equity — strategic-control	TBD (DD)	Majority	TBD	est.	FY27	A
5	MVJM Hospital	Perumbavoor, Ernakulam	~100	Family (verify)	Equity — strategic-control	TBD (DD)	Majority	TBD	est.	FY27	A
6	Nila Hospital	Thrissur	~80	Pvt (verify)	Equity — strategic-control	TBD (DD)	Majority	TBD	est.	FY28	A
7	MGM Clinics	Ernakulam Outskirts	~40	Family (verify)	Equity — bolt-on	TBD (DD)	Majority	TBD	est.	FY27	A
8	Aswani Hospital	Thrissur	300	Pvt (verify)	O&M concession	capex TBD	n/a	TBD	est.	FY27	B
9	Puthiyedom Hospital	Pala, Kottayam	~75	Family (verify)	O&M concession	capex TBD	n/a	TBD	est.	FY28	B
10	SR Medical College	Varkala, Trivandrum	300	Society — med college	O&M (30-yr)	capex TBD	n/a	TBD	est.	FY28	B
11	Wellmount Clinic	Kaloor, Ernakulam	Clinic	Pvt (verify)	O&M concession	capex TBD	n/a	TBD	est.	FY27	B
12	Chingavanam OP Clinic	Chingavanam, Kottayam	—	Family (verify)	Clinic / outpost	TBD	n/a	TBD	est.	FY27	B
13	PRV Medical Center	Thamanam, Kochi	—	Family (verify)	Clinic / outpost	TBD	n/a	TBD	est.	FY27	B

4D. Expanded Target Universe — Full Long List

The table below covers other hospitals across Kerala's 14 districts reviewed in the Brownfield DD process.

#	Hospital	District	Beds	Ownership	Posture	Deal Type	DD Notes
1	Renai Medicity	Ernakulam	335	Family/Promoter	Distressed	Equity 65%	CARE B; issuer non-cooperating since 2022; 5-sibling partnership sign-off required
2	Sunrise Hospital	Ernakulam	200	Family/Promoter	Conditional	Equity 70%	Multi-unit network; confirm carve-out vs chain. IVF licence portability.
3	Lourdes Hospital	Ernakulam	600	Mission/Trust	Conditional	O&M concession (25-yr)	NABH-accredited; 1,700 OPD/day; Latin Catholic; Archbishop engagement required.
4	Lisie Hospital	Ernakulam	1,500	Mission/Trust	Conditional	CoE / service-line tie-up	Syro-Malabar Archdiocese. 1,500 beds (NOT 600). Full oncology depth — CoE carve-out not appropriate. Tier-2 anchor candidate.
5	PVS Memorial Hospital	Ernakulam	310	Family/Promoter	NCLT — CIRP	NCLT resolution / bilateral	SC challenge on CIRP pending. Bilateral with P.V. Nidhish if SC upholds plan.
6	Specialists Hospital	Ernakulam	200	Family/Promoter	Conditional	Equity	Founder-doctor Dr K.R. Rajappan (Plastic Surgery). Single-founder dependency. EV ~₹100–150 Cr.
7	Welcare Hospital	Ernakulam	120	Family/Promoter	Open	Equity	Tier-3 suburban spoke; direct outreach to identify promoter.
8	Mar Baselios Medical Mission	Ernakulam	200	Mission/Trust	Open	Clinical affiliate	Idukki-border gateway. Tier-3.
9	MOSC Medical College	Ernakulam	700	Mission/Trust	Open	Clinical affiliate	Malankara Orthodox Church; 700 beds; medical college. Politically sensitive intra-church litigation.
10	Caritas Hospital	Kottayam	655	Mission/Trust	Conditional	O&M (25-yr)	Knanaya Catholic. Matha merged Sept 2024 — combined ~800 beds. Archbishop + Patron approval required.
11	Bharath Hospital	Kottayam	175	Charitable Society	Conditional	O&M (reclassified)	Charitable Society — NOT equity-acquirable. 175 beds (NOT 300). O&M concession, not equity.
12	Pushpagiri Medical College	Pathanamthitta	678	Mission/Trust	Conditional	JV 51:49	Academic JV. 100 MBBS seats. NMC/KUHS layers. Deadlock protection required in SHA.

#	Hospital	District	Beds	Ownership	Posture	Deal Type	DD Notes
13	Believers Church MC	Pathanamthitta	743	Mission/Trust	Conditional	CoE → O&M (optionality)	FCRA ban 2017; IT raids 2020; founder died May 2024. New Metropolitan Samuel Theophilus. 6-month clean-period required.
14	St. Gregorios Medical Mission	Pathanamthitta	250	Mission/Trust	Open	O&M / concession	Orthodox pilgrimage catchment (Parumala shrine). Tier-2.
15	Indo-American Hospital	Alappuzha	205	Charitable Foundation	Conditional	Equity / amalgamation (85%)	Founder ~94, Buffalo NY. Neuro-oncology bolt-on. Verify founder status Day-1.
16	Bishop Benziger Hospital	Kollam	600	Charitable Society	Conditional	O&M (20-yr)	Latin Catholic (NOT CSI). 600 beds (NOT 300). Capex ₹150–200 Cr. Bishop Mullassery engagement.
17	Holy Cross Hospital	Kollam	200	Mission/Trust	Conditional	O&M / concession	Carmelite mission. NH-66 location. Tier-2.
18	Upasana Hospital	Kollam	350	Family/Promoter	Conditional	Equity	Owned by Ravi Pillai (RP Group, Dubai-based). UAE-to-UAE conversation channel. EV ~₹150–200 Cr.
19	Cosmopolitan Hospital	Thiruvananthapuram	360	Family/Promoter	Conditional	Equity (backup to SUT)	8 directors — no single controller. EBITDA declining 14.7% YoY (pricing leverage). EV ₹250–400 Cr.
20	GG Hospital (Sree Gokulam)	Thiruvananthapuram	350	Family/Promoter	Conditional	Equity (backup to SUT)	New (2016); Gokulam Gopalan-backed. Diversified group — healthcare possibly non-core.
21	SUT Royal Hospital (TIHSL)	Thiruvananthapuram	400	Distressed / NCLT	NCLT — Liquidation	NCLT going-concern	Liquidation ordered Jun 2022. Liquidator: Raju Kesavan. Competing bidder risk.
22	PRS Hospital	Thiruvananthapuram	350	Family/Promoter	Conditional	Equity	Ortho/spine franchise. Tier-2.
23	Mother Hospital	Thrissur	300	Family/Promoter	Conditional	Equity 80–100%	Second-generation family. Clean compliance. EV ₹300–450 Cr. Central Kerala anchor.
24	West Fort Hi-Tech	Thrissur	300	Family/Promoter	Conditional	Equity	Fallback if Mother unavailable. Tier-2.
25	Amala Institute (Cancer)	Thrissur	700	Mission/Trust	Conditional	CoE carve-out	CMI Devamatha Province. Established cancer centre — oncology partnership fit. Tier-2.

#	Hospital	District	Beds	Ownership	Posture	Deal Type	DD Notes
26	Karuna Medical College	Palakkad	800	Family/Promoter	Open	Equity	Underserved district. Medical college. Tier-2.
27	Iqraa International	Kozhikode	500	Mission/Trust	Conditional	O&M / concession	Only large independent option in Kozhikode (BMH/KKR, Meitra/KKR, Aster MIMS taken). Tier-2 defensive.
28	AKG Memorial Co-op	Kannur	500	Cooperative	Conditional	Management contract	Co-op governance (AGM vote required). CPI-M political ecosystem. Airport-corridor NRI catchment.
29	Daya General Hospital	Thrissur	500	Family/Promoter (public ltd)	Conditional	Equity 75%	FY25 rev ₹187 Cr, 15.9% EBITDA. Public Ltd, ~17-director board — share aggregation for control. ~7× EBITDA (~₹210 Cr).
30	Ananthapuri Hospitals	Thiruvananthapuram	400	Family/Promoter	Conditional	Equity 75%	FY25 rev ₹175 Cr, 13.4%. Trivandrum tertiary; capital-region anchor alongside SUT Royal. ~₹165 Cr.
31	Dr. KM Cherian IMS	Alappuzha	300	Family/Promoter (Pvt Ltd)	Conditional	Equity 75%	FY25 rev ₹168 Cr, 17.0% (best margin). Chengannur cardiac tertiary; Christian-founded but Pvt Ltd → equity, not O&M. ~₹200 Cr.
32	Starcare Hospital	Kozhikode	290	Family/Promoter	Conditional	Equity 75%	FY25 rev ₹106 Cr, 12.9%. First Malabar foothold (KKR/Aster-contested). ~₹95 Cr.
33	Mother Care & Health Centre	Kozhikode	250	Family/Promoter	Conditional	Equity 75%	FY25 rev ₹63 Cr, 11.2%. W&C / maternity; pairs with Starcare. ~₹50 Cr.
34	Meditrina Hospitals	Kollam	175	Family/Promoter	Conditional	Equity 75%	FY25 rev ₹181 Cr, 11.1%. Cardiac franchise; confirm carve-out from multi-state network. ~₹140 Cr.
35	Avitis Super Specialty	Palakkad	180	Family/Promoter	Conditional	O&M / turnaround	(O&M). FY25 rev ₹69 Cr but LOSS (-4.5%). Managed turnaround, not equity. Opens Palakkad. ~₹50 Cr capex.
36	Malabar Hospitals	Kozhikode	100	Family/Promoter	Open	Equity (watch)	Universe / watch. FY25 rev ₹43 Cr, 15.4%. Smallest; Malabar contested — not top tier.

PART IV FINANCIALS & RETURNS

5. Financial Model & Returns

The plan estimates a total programme envelope of about ₹3,900 crore with roughly 55% from VPS Healthcare or Burjeel equity and 45% from debt. On a five-year hold the base case returns about 2.4 times the equity invested (roughly a 20% annual return). In a genuine downside with possibly slower deleveraging and weaker EBITDA, the program can return less than 1.5 times the capital (~1.4x); a stronger case more than triples it (~3.5x). The purpose of the programme is scale and a defensible regional position, so the upside from referral growth, O&M performance fees and a higher exit valuation is deliberately left out of these base-case numbers.

₹ Crore (VPS economic share)	FY26	FY27	FY28	FY29	FY30
Network revenue	265	950	1580	1950	2090
Network EBITDA (~24% margin)	63	223	373	462	495
Net profit	8	37	91	166	203
Free cash flow	0	12	63	153	206

Five-year growth snapshot. Revenue grows ~18x (₹115 Cr → ₹2,090 Cr) and EBITDA ~17x (₹29 Cr → ₹495 Cr). Managed (O&M) hospitals add fee income only — their full hospital revenue is not counted here, and at exit the fee stream is valued as a services annuity (8x), not at a hospital multiple (14x). Because an O&M concession is terminable, the 8x services-annuity multiple is itself an assumption.

Key Model Assumptions

Parameter	Value	Basis
Hospital revenue ramp (3-yr CAGR)	10% p.a.	Utilisation + ARPP uplift during turnaround; benchmarked to VPS Lakeshore trajectory
O&M management fee	6.5% of gross hospital revenue	Midpoint of 5–8% range
VPS EBITDA margin on O&M fee income	~55%	Net of deployed CEO / CFO / Clinical Head cost at the institutions
Corporate tax rate	25.17%	Finance Act 2020 new regime incl. surcharge
D&A (% of Owned+JV revenue)	3.0%	Hospital fixed assets only; not O&M fee income
Maintenance capex (% of Owned+JV revenue)	2.5%	Recurring; excludes growth capex
Exit multiple — Owned hospital EBITDA	14x (base)	20% discount to listed Kerala peer multiples of 18–25x
Exit multiple — O&M fee-stream EBITDA	8x (base)	Services annuity — NOT a hospital multiple
Blended debt cost	9.5%	Indicative healthcare term-loan pricing, May 2026
Programme envelope	₹3,900 Cr	

Parameter	Value	Basis
Equity (VPS/Burjeel accruals)	₹2,150 Cr (55%)	Subject to parent funding bridge
Debt facility	₹1,750 Cr (45%)	Staggered draws: 29% FY26 / 53% FY27 / 18% FY28

Per-Hospital Economics (FY30 Steady-State)

Hospital	Treatment	Entry Rev	Entry EBITDA%	VPS Stake	Mature Hosp Rev	Mature Econ Rev	Mature Econ EBITDA	Y3 Target Margin
Renai Medicity	OWNED	₹160 Cr	8%	65%	₹213 Cr	₹138 Cr	₹30 Cr	22%
Sunrise Hospital	OWNED	₹120 Cr	16%	70%	₹160 Cr	₹112 Cr	₹25 Cr	22%
Lourdes Hospital	MANAGED (O&M fee)	₹230 Cr	15%	n/a	₹306 Cr	₹19.9 Cr (fee)	₹10.9 Cr	24% (hosp margin)
Caritas Hospital	MANAGED (O&M fee)	₹180 Cr	17%	n/a	₹240 Cr	₹15.6 Cr (fee)	₹8.6 Cr	22% (hosp margin)
Pushpagiri Med. College	JV (51%)	₹210 Cr	20%	51%	₹280 Cr	₹142.6 Cr	₹34.2 Cr	24%
Indo-American Hospital	OWNED	₹90 Cr	18%	85%	₹120 Cr	₹101.8 Cr	₹22.4 Cr	22%
Bishop Benziger Hosp.	MANAGED (O&M fee)	₹90 Cr	13%	n/a	₹120 Cr	₹7.8 Cr (fee)	₹4.3 Cr	22% (hosp margin)
SUT Royal (TIHSL)	OWNED	₹97 Cr	4%	100%	₹129 Cr	₹129.1 Cr	₹31.0 Cr	24%
Mother Hospital	OWNED	₹150 Cr	18%	90%	₹200 Cr	₹179.7 Cr	₹43.1 Cr	24%
Orchid Medicity	OWNED	₹110 Cr	12%	75%	₹146 Cr	₹110 Cr	₹22 Cr	20%
Sreevalsam IMS	JV (51%)	₹120 Cr	10%	51%	₹160 Cr	₹81 Cr	₹16 Cr	20%
CM Hospital	OWNED	₹50 Cr	11%	75%	₹67 Cr	₹50 Cr	₹10 Cr	20%
MVJM Hospital	OWNED	₹40 Cr	12%	75%	₹53 Cr	₹40 Cr	₹8 Cr	20%
Joys Hospital	OWNED	₹35 Cr	13%	80%	₹47 Cr	₹37 Cr	₹8 Cr	22%
Aswani Hospital	MANAGED (O&M fee)	₹150 Cr	14%	n/a	₹200 Cr	₹13.0 Cr (fee)	₹7.1 Cr	22% (hosp)
SR Medical College	MANAGED (O&M fee)	₹110 Cr	10%	n/a	₹146 Cr	₹9.5 Cr (fee)	₹5.2 Cr	20% (hosp)

Hospital	Treatment	Entry Rev	Entry EBITDA%	VPS Stake	Mature Hosp Rev	Mature Econ Rev	Mature Econ EBITDA	Y3 Target Margin
Puthiyedom Hospital	MANAGED (O&M fee)	₹30 Cr	11%	n/a	₹40 Cr	₹2.6 Cr (fee)	₹1.4 Cr	20% (hosp)
Travancore Medical College	MANAGED (O&M fee)	₹180 Cr	10%	n/a	₹240 Cr	₹15.6 Cr (fee)	₹8.6 Cr	20% (hosp)
Daya General	OWNED	₹187 Cr	16%	75%	₹249 Cr	₹187 Cr	₹42 Cr	22%
Ananthapuri	OWNED	₹175 Cr	13%	75%	₹233 Cr	₹175 Cr	₹39 Cr	22%
Dr. KM Cherian	OWNED	₹168 Cr	17%	75%	₹223 Cr	₹168 Cr	₹37 Cr	22%
Starcare	OWNED	₹106 Cr	13%	75%	₹141 Cr	₹106 Cr	₹23 Cr	22%
Mother Care	OWNED	₹63 Cr	11%	75%	₹84 Cr	₹63 Cr	₹14 Cr	22%
Meditrina	OWNED	₹181 Cr	11%	75%	₹241 Cr	₹181 Cr	₹41 Cr	22%
Avitis (turnaround)	MANAGED (O&M fee)	₹69 Cr	(4)%	n/a	₹92 Cr	₹6.0 Cr (fee)	₹3.3 Cr	20% (hosp)
NETWORK TOTAL		₹3,101 Cr				₹2,090 Cr	₹495 Cr	23.7%

Equity Returns — Scenario Analysis

	Bear	Base	Bull
FY30 EBITDA (₹ Cr)	₹420 Cr	₹495 Cr	₹555 Cr
Exit Enterprise Value	₹4,390 Cr	₹6,370 Cr	₹8,490 Cr
Less: Net Debt at Exit	(₹1,400 Cr)	(₹1,150 Cr)	(₹950 Cr)
Exit Equity Value	₹2,990 Cr	₹5,220 Cr	₹7,540 Cr
Total Equity Invested	₹2,150 Cr	₹2,150 Cr	₹2,150 Cr
Equity IRR (5-yr approx)	~7%	~20%	~29%
MOIC (gross)	~1.4x	~2.4x	~3.5x

Risk	Severity	Description	Mitigant
Capital mobilisation	High	₹2,150 Cr equity commitment requires Burjeel Holdings balance-sheet headroom alongside ongoing capex; bridge financing terms not yet finalised.	Board-level capital allocation resolution + 5-year consolidated cash-flow model before FY27 deployment; phased drawdown tied to due-diligence milestones.
Regulatory & licensing transfer	High	Key statutory licences (AERB, CEA, THOA, ART/IVF) held by incumbent institutions; transfer delays or denials halt revenue-generating services and affect deal completion.	Full licence mapping as condition precedent to signing; specialist regulatory counsel engaged pre-LOI; transfer timeline embedded in integration schedule.
Execution & integration complexity	Medium	Simultaneous management of acquisitions, JV concessions, and O&M contracts across multiple sites creates bandwidth, governance, and physician-retention risks.	Dedicated integration PMO from Day 1; phased deployment with IC gate reviews; retention provisions in all O&M agreements.
Exit liquidity & multiple compression	Medium	Kerala hospital M&A market is thin; bear-case exit at 11x EBITDA yields sub-3% IRR. Sector PE multiples are sensitive to broader healthcare liquidity cycles.	Three parallel exit routes prepared from Year 3: strategic trade sale, secondary PE sale, and Burjeel Holdings group consolidation or IPO.

PART V STRATEGY & NETWORK

7. Kerala Expansion Playbook

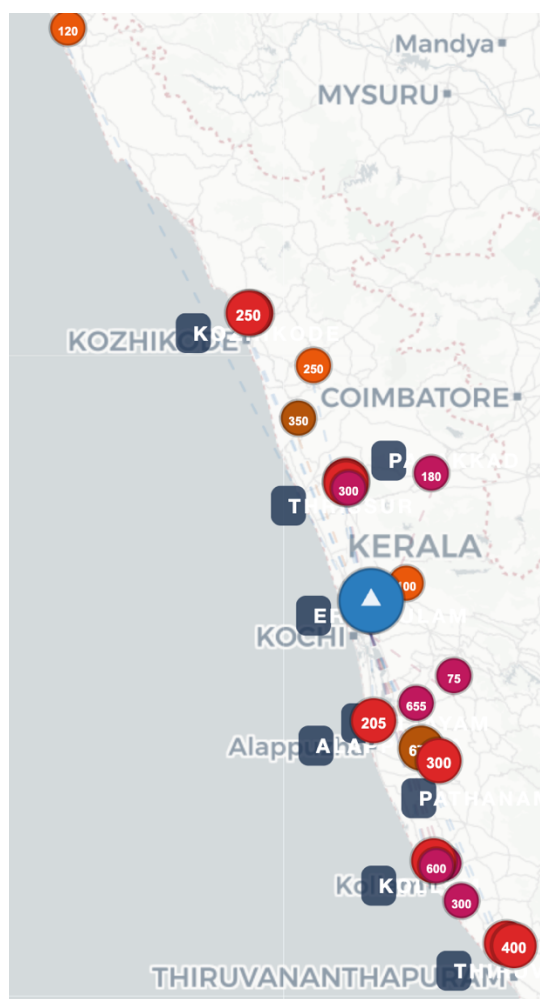
Master Strategy · Entry Playbooks · District Map · Phasing

Strategic Frame

Kerala's hospital market is consolidating around branded, accredited networks. VPS's winning position is **clinical differentiation at the VPS Lakeshore quaternary hub + speed in Tier-2 acquisition and mission-hospital O&M** — not out-scaling entities that are backed by PE capital. The program is a hub-and-spoke build using pre-defined prototypes.

State Map — Base-Case Geography

Exhibit · Project Nila network — city-level views of the base-case network feeding the VPS Lakeshore quaternary hub at VPS Lakeshore (north to south).



8. Expanding the Network Architecture

The Seven Prototypes

Prototype	Description	Capital Model	Candidates
P1 — Precision Medicine Centre	Kochi New Block: genomics-integrated oncology, BMT/CAR-T, transplant, rare disease, clinical trials.	₹300–500 Cr greenfield	Lakeshore VPS Lakeshore New Block and Delhi precision Medicine Centre (in planning)
P2 — Superspecialty Hub	Kochi Main Block (existing VPS Lakeshore): cardiac, neuro, MCH, ortho, GI, transplant; quaternary referral.	Operational — maintenance capex	Kochi Main Block , SUT Royal Trivandrum, Renai, Sunrise
P3 — Urban Multispecialty	IT-corridor / urban professional catchment; gut health, MSK, longevity verticals.	₹50–375 Cr per acquisition	VPS Kakkanad, Mother, Indo-American, Cosmopolitan
P4 — Embedded Partner Unit	Lakeshore-branded floor/wing within a partner hospital; VPS clinical protocols and staff.	₹10–30 Cr per unit	Lisie (service-line), MOSC Medical College
P5 — Risk Assessment Kiosk	High-footfall cancer/cardiac/genomic risk triage; EMR-linked, 24-hr follow-up.	₹2–5 Cr per kiosk	Airports, malls, corporate parks — Gulf-corridor hubs
P6 — Whole-Hospital O&M	Full operational authority under 15–25 yr concession; fee of 5–8% gross revenue + EBITDA-share.	₹50–150 Cr concession capex	Lourdes, Caritas, Bishop Benziger; St. Gregorios, Holy Cross, Iqraa
P7 — Academic Health JV	51:49 JV with medical-college trust; VPS runs clinical operations; trust retains academic licensing.	₹100–150 Cr JV capex	Pushpagiri, Amala (Thrissur), Jubilee Mission

Future Phase Expansion Pipeline

Post-FY28, the network expansion follows three vectors:

- **O&M pipeline expansion:** Target 1–2 additional concessions before exit (St. Gregorios Medical Mission Parumala, Holy Cross Kottiyam, Iqraa International Kozhikode, AKG Memorial Co-op Kannur). Each additional O&M concession adds ~₹5–10 Cr fee EBITDA at exit — additive to base-case services multiple.
- **Tier-2 equity add ons:** West Fort Hi-Tech (Thrissur), Upasana Hospital (Kollam / Ravi Pillai's RP Group), PRS Hospital (Trivandrum ortho/spine), Karuna Medical College (Palakkad, underserved district). Each adds spoke capacity at 40–50% below greenfield replacement cost.
- **P5 Risk Kiosks:** Asset-light deployment at Cochin International Airport, Trivandrum Airport, Kozhikode Airport and select malls in the Gulf corridor. Capital per kiosk ~₹2–5 Cr; drives VPS Lakeshore referral pipeline from high-footfall NRI touchpoints.

PART VI DEAL EXECUTION

9. IB Execution Guide

Five-Phase Deal Process

Phase	Activity	Gate
1 — Origination	Target list; independently verify ownership/NCLT status of every lead before any outreach — incl. intermediary-introduced leads.	No outreach until ownership verified.
2 — Approach	NDA; indicative interest letter; preliminary desk DD.	Signed NDA.
3 — LOI	Non-binding LOI / term sheet; exclusivity where possible; Board-noted.	Board-noted LOI.
4 — Diligence	Full DD under NDA: QoE; title; licences; doctor retention; Sec 29A (NCLT assets).	Clean DD + walk-away price.
5 — Close	SPA / concession agreement; CCI/FEMA filings; financing CPs; 100-day plan signed off.	Signed; all conditions precedent met.

DD Workstreams

Critical items must close before SPA / concession execution. Broad Level workstreams are below.

Workstream	Critical Items	Applies To
Financial	Normalised EBITDA FY23–FY25; revenue quality; related-party transactions; D&A and capex schedule	All targets
Clinical & HR	Consultant contracts; key-man dependency; NABH accreditation status; doctor retention risk	All targets
Regulatory & Licensing	THOA, AERB, CEA, ART/IVF portability on change of control; NABH validity; MCI/NMC	All targets, especially those with transplant, radiotherapy, IVF, or medical college
Legal / Title	Title/encumbrance on all property; Sec 29A eligibility (NCLT targets); CCI threshold assessment; FEMA/FDI route	All targets
O&M Legal	Concession enforceability over charitable/trust property; ability to bind successors; Charity Commissioner requirement; tax treatment of management fee (GST)	All O&M targets

Before any binding offer, the programme runs a structured due-diligence process across eleven workstreams, targeting a close by October 2026.

Workstream	Items	Critical	Key risks to clear
A. Commercial & Market	7	4	FY27 revenue bridge; doctor retention at acquired Kerala hospitals
B. Financial	11	6	Debt consents; PMJAY claim settlement lag; undisclosed liabilities
C. Legal & Corporate	11	6	CCI filing timing; Burjeel FDI/FEMA route; Burjeel FDI route
D. Clinical & Operational	9	4	AERB licence validity; LINAC downtime risk; medical physicist shortage
E. Human Resources	6	1	oncologist retention; PF/gratuity liabilities; union presence
F. IT & Systems	3		Lakeshore-target HIS integration complexity
G. Real Estate & Infra	5	1	Kerala greenfield land conversion; Kerala greenfield site geotech
H. Regulatory & Licensing	7	1	AERB for all radiotherapy units in target hospitals; Kerala CEA renewals
I. Insurance & Indemnity	2		LINAC breakdown cover adequacy; professional indemnity gaps
J. ESG & Environmental	3		Biomedical waste compliance at smaller centres
K. Deal Structure & Integration	7	3	Bank term sheet execution; Lakeshore board composition / consent

Advisor Mandate – Next 60 days

- Legal DD Counsel (AZB, Cyril Amarchand, or Trilegal) — lead legal due diligence across the target shortlist: title searches, NCLT/CIRP status and Sec 29A clearance, promoter litigation screening, concession enforceability over trust property, and security structure for O&M capex.
- Financial DD / QoE Advisor (Big-4: PwC or EY) — quality-of-earnings, working-capital normalisation, and debt-mapping on top-5 priority targets; independent revenue audit for Renai Medicity; EBITDA adjustability and capex liability assessment.
- M&A / Investment Banker (healthcare-specialist; e.g. Kotak, JM Financial) — run structured deal processes, manage seller outreach, and provide valuation benchmarking across all shortlisted targets.
- Regulatory & Tax Counsel — review optimal acquisition structures (direct purchase, slump sale, SPV, or O&M concession) per target archetype; stamp duty, GST, and income-tax implications; FCRA/FEMA treatment for mission-hospital channel; Burjeel → VPS Lakeshore → SPV FDI route and RBI reporting.
- Operational DD Lead (internal team) — assess clinical governance, staffing structures, bed-utilisation patterns, and NABH/JCI compliance gaps across concession targets.
- Debt Financing Advisor / Arranger (SBI Capital, ICICI Securities, or Axis Capital) — structure the ₹1,750 Cr debt envelope: term-sheet parameters, security package, and covenant framework.

PART VII VALUE CREATION PLAN

10. Value Creation Plan

Value Creation Levers

Revenue Growth — Utilisation & ARPP Uplift

- **Occupancy recovery to 70–80%.** Entry levels (SUT 4%, Renai 8%, Bishop Benziger 13%) are well below peer norms of 65–75%; VPS Lakeshore referral protocols and TPA empanelment are the primary levers.
- **ARPP expansion via CoE routing.** Routing high-value cardiology, oncology, and neuroscience cases to hub hospitals lifts blended ARPP without additional beds.
- **Payor mix shift.** Extend VPS Lakeshore's corporate/TPA panel across the network — insurance mix from 20–35% at entry to 40–55% by Year 3.
- **O&M fee leverage.** The 6.5% management fee on gross revenue means every ₹10 Cr of growth across Lourdes, Caritas, and Bishop Benziger adds ~₹65 L directly to VPS fee income.

Margin Expansion

- **Workforce right-sizing: shift visiting consultants to full-time VPS-network physicians; renegotiate visiting packages using network volume** as leverage.
- **Central procurement:** pharma and consumables via VPS Lakeshore vendor relationships — 3–5% COGS savings (~₹8–15 Cr by FY28).
- **Shared services:** Finance, HR, IT, and biomedical maintenance centralised at VPS Lakeshore — ~₹8–12 Cr overhead savings at run-rate (FY28+).
- **NCLT reset:** Entities like SUT have a clean-slate NCLT entry with no legacy liabilities; 100-day plan targets 24% EBITDA margin by Year 3 from a 4% base.

100-Day Plans by Archetype

Owned Acquisitions (Renai, SUT Royal, Sunrise, Indo-American, Mother)

Days 1–30: Stabilise & Assess

- Appoint VPS-nominated CEO/COO and CFO at each Owned hospital within 30 days; retain incumbent clinical management for continuity.
- Sign retention packages for top-20 revenue consultants; priority at NCLT centres and Indo-American (neuro-oncology team).
- Operational audit: occupancy by specialty, payor mix, revenue cycle, pharmacy COGS, and vendor contracts.
- Daily revenue flash reporting across all Owned hospitals on VPS Lakeshore MIS format.

Days 31–60: Plan & Initiate

- Onboard all Owned hospitals onto VPS Lakeshore's pharmaceutical/consumable vendor framework.
- Enrol each hospital with 3+ new corporate/TPA accounts from VPS's panel; target ₹15–20 Cr incremental revenue in Year 1.

- Deploy VPS clinical protocols (infection control, antibiotic stewardship, clinical audit) across all Owned hospitals.
- Look at COGS, staff , pharmacy and vendor related cost savings.
- Begin NABH re-accreditation if needed.

Days 61–100: Execute & Measure

- First monthly operating review: CEO presents revenue, occupancy, ARPP, EBITDA vs plan.
- Implement performance-linked compensation for HODs tied to EBITDA and patient-satisfaction.
- Shared-services feasibility study: identify Finance, HR, IT, and biomedical functions for FY27 centralisation.

O&M Concessions (Lourdes, Caritas, Bishop Benziger)

- Deploy O&M team (CEO/COO, CFO, Clinical Head); baseline audit to align revenue definition, billing codes, and reconciliation methodology with trust board.
- Identify top-10 revenue consultants; design retention plan within VPS framework under trust payroll.
- VPS revenue-cycle rollout: standardised billing and pre-authorisation workflows — typically 3–5% revenue uplift from billing-leakage recovery within 60 days.
- First management fee: reconcile, invoice, and collect Month 1/2 fee from each trust; present 100-day review to trust board.

KPI Dashboard

KPI	Entry Baseline	Year 1 Target	Year 3 Target
Network Econ Revenue (₹ Cr)	265	950	1,580 (FY28)
Network EBITDA (₹ Cr)	63	223	373 (FY28)
EBITDA Margin	23.8%	23.5%	23.6%
O&M Fee Income (₹ Cr)	~8.5	~31	~62 (FY28)
Net Debt / EBITDA (x)	13.5x	9.7x	5.6x (FY28)
Bed Occupancy (%)	45–65%	65–70%	70–80%
Average Revenue Per Patient — IP	₹28,000–45,000	₹35,000–50,000	₹38,000–55,000
Insurance/Corporate Payor Mix	20–35%	35–45%	40–55%
Consultant Retention Rate (%)	—	>90%	>95%
NABH Accreditation Status	Varies	All applied	All accredited

PART VIII STRATEGIC QUESTIONS

11. Key Due-Diligence Questions

The following questions constitute the critical open items to be resolved through due diligence before any Board approval, LOI, or capital deployment. Each advisor mandate maps directly to one or more of these questions.

A. Financial & Returns

- Is the base-case 20% IRR / 2.4× MOIC acceptable as a strategic-platform return, or does the programme require a higher-return subset of targets to proceed?
- What is the normalised EBITDA and working-capital position for each Tier-1 equity target, which can be validated by independent QoE?
- What is the verified free cash flow available from VPS Lakeshore and Burjeel Holdings to fund the ₹2,150 Cr equity envelope without external dilution?
- Do the financial projections for NCLT assets like SUT Royal (4% → 24% EBITDA in 3 years) hold under independent operational review?

B. Legal, Regulatory & Structural

- Does VPS pass Sec 29A eligibility for the SUT Royal NCLT bid, and is specialist insolvency counsel retained?
- Can all critical licences (AERB, CEA, THOA, ART/IVF) be transferred or re-issued in VPS's name, and on what timeline?
- Is the Burjeel → VPS Lakeshore → SPV FDI structure cleared for FEMA/RBI compliance and ADX related-party disclosure?
- DO the entities of specific targets have an ownership and title position that support a clean acquisition, and can FY23–25 audited accounts be obtained as a condition precedent?

C. O&M Channel Credibility

- Can a Diocese or archdiocese board legally enter and bind successors to a 15–25-year O&M concession agreement and what are the charity commissioner rules around this?
- Has at least one LOI or term sheet with any potential target been executed before taking it to the Board?
- What are the physician retention provisions in each O&M agreement, and do they hold under trust employment?

D. Execution & Governance

- Is CCI counsel's opinion received on individual deal notification thresholds and programme-aggregate related-transaction exposure?
- Is the phased deployment sequenced such that Tranche 2 commitments are conditioned on Tranche 1 operational delivery?

PART IX COMPETITIVE INTELLIGENCE

12. Competitive Landscape — India Hospital Sector

The Kerala Competitive Map

Actor	Kerala Footprint	Strategy	Relevance to VPS
Aster DM Quality Care (Aster + QCIL merger; closes Q1 FY27)	Aster: Medcity Kochi (~900 beds), MIMS Kozhikode, MIMS Kasaragod (264 beds, Oct 2025), Aster Capital Trivandrum (454 beds, planned). KIMS (via QCIL): Trivandrum, Kottayam, Kollam, Perinthalmanna (~1,475 beds)	Merged entity: 39 hospitals, 28 cities, 10,625+ beds. ₹4,000 Cr capex plan; 4,080 beds in pipeline. Focus on brownfield expansion and synergy capture post-merger.	Dominant in Kochi, North Kerala, and South Kerala urban centres. Concentrating on owned assets and brownfield while not pursuing O&M concessions. Mission-hospital channel remains open.
KKR / Baby Memorial Hospital (BMH)	BMH flagship, Kozhikode (~600 beds); Kannur; Meitra Hospital, Kozhikode (230 beds, acquired Sep 2025). Total: 9 hospitals across Kerala, Tamil Nadu, Telangana; ~3,000 beds.	Buy-and-build: KKR invested ₹5,100+ Cr since Jul 2024 (70%→75% stake). Acquired Meitra (2025) and Star Hospitals, Hyderabad (2025). Targeting 3,000 beds; Chennai expansion underway.	Concentrated in North Kerala. Acquisition-led model competes for the same distressed and succession assets. Not active in O&M.
Apollo Hospitals	No material Kerala presence.	National scale; selective entry via acquisition.	Low near-term. Could enter via a single large Kerala acquisition (e.g. Medical Trust, Lisie). Same target pool as VPS.
Manipal Hospitals (TPG-backed)	No material Kerala presence.	Pre-IPO. Karnataka base. Acquired Sahyadri Hospitals (Mumbai, Jul 2025, ~\$760 Mn). Selective pan-India expansion.	Low near-term. Likely enters Kerala via a distressed or succession asset — overlaps with VPS target set on exit.
Narayana Health	No Kerala presence.	Volume-led, price-competitive model. Karnataka and East India focus.	Negligible. Model does not align with Kerala's payor mix.

Competitive Implications for VPS Strategy

- Aster–QCIL's merger concentrates scale in Kochi and North Kerala's premium urban segment. The combined entity (39 hospitals, 10,625+ beds, ₹4,000 Cr capex plan) operates primarily through owned and brownfield assets.
- No active competitor is pursuing O&M concessions with Catholic or Orthodox mission hospitals. Aster, KKR/BMH, and Manipal operate through equity acquisitions, which mission-hospital governance structures typically don't allow.
- SUT Royal (Kochulloor, Trivandrum) is in active NCLT liquidation. Cannot miss this process so urgency is key.
- Succession-stress across Tier-1 targets is concentrated in the next 24–36 months; seller motivation is likely to reduce materially beyond that horizon.

Institutional Capital Activity in Indian Healthcare

Institution / Strategic Actor	Recent Move	Relevance to VPS
KKR / Baby Memorial Hospital (BMH)	Acquired 70% of BMH (Kozhikode) in Jul 2024 for ₹2,100 Cr; increased stake to ~75%	Primary active competitor in Kerala acquisitions. North Kerala tier locked.

	via ₹1,750 Cr rights issue (Apr 2026). Total KKR commitment: ₹5,100+ Cr. Acquired Meittra Hospital (Kozhikode, Sep 2025) and 60% of Star Hospitals (Hyderabad). Network: 9 hospitals, ~3,000 beds across Kerala, Tamil Nadu, Telangana.	Competes for succession and distressed assets in the same pool.
Aster DM Quality Care (post-Aster + QCIL merger)	Merger announced Nov 2024; shareholder approval Mar 2026; expected close Q1 FY27. Combined entity: 39 hospitals, 28 cities, 10,625+ beds. ₹4,000 Cr capex plan; 4,080 beds in pipeline.	Dominant in Kochi and South Kerala urban. Post-merger integration focus limits near-term appetite for additional acquisitions.
IHH Healthcare (Malaysia)	Controls Fortis Healthcare (pan-India). No direct Kerala presence.	Potential exit counterparty for a South India hospital platform.
Temasek	Incremental stakes across multiple Indian hospital chains.	Financial investor. Possible co-investor if VPS pursues a structured equity raise.
ADIA / GIC	Healthcare infrastructure investments across India.	Gulf-linked sovereign investors; natural fit for a Burjeel / VPS UAE–India story.
Manipal Hospitals (TPG-backed)	Pre-IPO. Acquired Sahyadri Hospitals (Maharashtra) for ~\$760 Mn (Jul 2025). Karnataka base.	No current Kerala presence. Most likely to enter via a distressed or succession asset — overlaps with VPS target set as a potential exit buyer.